STANDARD OPERATING PROCEDURE			P-3433
	SARPY COUNTY SHERIFF'S OFFICE PAPILLION * NEBRASKA GREG LONDON SHERIFF	DATE ISSUED 06/01/06	DATE REVISED **06/08/22**	DATE REVIEWED 02/02/25
		APPROVED: *GREG LONDON*		
CHAPTER TITLE ROAD PATROL	RESOURCES **N.R.S. 23-1210, 23-1801-23-1821**			
SUBJECT CORONER CALL	CROSS REFERENCE P-3431 DEATHS P-4010 SERIOUS PERSONAL INJURY AND FATALITY CRASHES G-6200 EVIDENCE AND PROPERTY I-3110 ON-CALL INVESTIGATOR I-3300 DEATH INVESTIGATION			

I. Purpose

The purpose of this policy is to establish guidelines for road patrol supervisors regarding conducting thorough coroner investigations.

II. Policy

The Sarpy County Sheriff's Office, as designated by the Sarpy County Attorney's Office (N.R.S. 23-1210 and 23-1817), has the authority of the county coroner. The on-duty road patrol supervisors shall have the responsibility of all hospice and comfort care coroner calls, along with deaths where a doctor will sign the death certificate within Sarpy County, regardless of law enforcement jurisdiction. The on-call Investigator shall have the responsibility of all non-hospice/comfort care coroner calls within Sarpy County, regardless of law enforcement jurisdiction, where a Doctor is unwilling to sign a death certificate. An on-duty road patrol supervisor shall respond to all deaths within the Sheriff's Office jurisdiction to determine if the person has died by unlawful means. If an on-duty road patrol supervisor is not available, he or she may designate an available deputy, with previous investigation division experience or the on-call investigator to respond. Generally, a coroner shall be requested after initial response by law enforcement and/or rescue personnel. This policy is specifically intended for road patrol supervisors functioning as coroners and not as a guideline for initial responding deputies to deaths or death investigations performed by investigations (see P-3431 Deaths and I-3300 Death Investigations for further details.) When a death occurs outside of Sheriff's Office jurisdiction that is not hospice or comfort care, the Road Patrol Supervisor is not required to respond. The on-call Investigator may be summoned by the investigating agency directly to respond to perform coroner duties.

III. Procedure

A. The following actions shall be taken by the road patrol supervisor or designee as applicable upon the receipt of a coroner call.

1. Contact the first law enforcement responder and obtain the following information.

- **a)** The location of the body
- **b)** Who discovered the body
- **c)** When was the body discovered
- **d)** Was the body moved. If so, why and by whom.
- **e)** Who, including the first law enforcement responder, has been in the room or area where the body is located

2. Assess crime scene security.

3. Identify and interview witnesses and obtain the following:

- **a)** Who was the reporting party
- **b)** What events occurred prior to 911 call
- **c)** When was the deceased last seen
- **d)** Victim's medical history and medications
- **e)** Victim's primary care physician

4. Photograph body, room or area, entrance and exits. Photographs do not need to be taken if the individual is under hospice care, comfort care, palliative care, or the death is of natural causes and the primary physician of the deceased is signing the death certificate.

5. Note condition of body.

- **a)** Blood located in or on the ears, nose, mouth, and/or torso
- **b)** Bruises and external injuries

6. Note body status:

- **a)** Temperature: The human body typically cools at an average of 1.5 degrees per hour. Typically, it takes the body 18-36 hours to cool to the ambient temperature. There are many variables that can affect the cooling rate, such as the temperature surrounding the body, air movement, original body temperature, body weight, body fat (more obese individuals cool more slowly), clothing, and the surface on which the body is lying.
- **b)** Livor Mortis (Lividity): Lividity is caused by the heart stopping, and the subsequent reduction in blood pressure.

Under the influence of gravity, the blood then slowly drains to the lower-lying parts of the body. On the surface of the skin, this shows as paleness to the upper portions of the body, and a dark redness on the lowest-lying parts of the body.

Lividity typically begins within 2 hours of death; sometimes it begins to show within minutes. Within 8-12 hours it becomes relatively fixed. In some cases, however, it will never become fixed. In 3-5 days, lividity may become permanent.

Lividity can look similar to bruising; however, bruising will not change color if pushed with a finger, lividity will if it is not fixed.

c) Rigor Mortis: Rigor mortis is caused by the muscles of the body being denied oxygen subsequent to the heart stopping. The muscles then stop producing Adenosine Triphosphate (ATP), which is used by the muscles to process glucose and actin. Once the ATP remaining in the muscles is exhausted, the fibrous cells of the muscles, which normally slide past each other freely, become adhered together by the glucose and actin. This produces the stiffening effect called rigor mortis.

Rigor mortis typically begins within 2-4 hours. It begins simultaneously in all the muscles, but is apparent in the smaller muscles first, such as the muscles of the jaw. It progresses through the body for a period of 6-8 hours. After approximately 36 hours, secondary flaccidity may take effect, caused by the deterioration of the muscle tissues. However, in some cases, secondary flaccidity may not take place at all.

Rigor mortis can be useful to the coroner in determining whether or not the scene or the body has been disturbed since the death. Rigor mortis usually does not change the position of the body. The body merely stiffens in the exact position in which it is lying. The body should be in a relaxed position consistent with the cause and mechanism of death.

d) Odor and decomposition: Putrefaction (the process of decaying) proceeds at a rate that varies with a number of factors. Temperature is a crucial factor; If the body is in a frozen environment; for instance, putrefaction may not take place at all. Putrefaction is a longer-term process than the

other post-mortem changes, and is even more dependent on the environment.

Within approximately two to three days, a greenish discoloration will be visible in the area of the cecum (bowels) and stomach. The body will begin to smell.

7. Clothing and Personal Property: Document clothing and personal property. The coroner shall be responsible for all personal property on the body. At the coroner's discretion, personal property may be released to family, Investigating Agency, or taken into safekeeping and receipted to the Sarpy County evidence custodian. The coroner shall insure that items found on the deceased are noted in the coroner's report. Personal property is the responsibility of the coroner and if the coroner takes items for safe-keeping an evidence form shall be completed. (see N.R.S. 23-1815 and G-6200 Property and Evidence.)

8. Inspection of Body: When viewing the body all persons, other than the coroner and one deputy, shall be asked to leave the room. No more then two should be present when the body is examined. In the event the incident occurs in a municipal area it shall be the coroner and one officer or detective from the municipality. The complete body shall be viewed and turned for complete inspection. Any peculiarities shall be photographed and noted.

9. Trace Evidence: The coroner shall be especially observant for any trace evidence at the scene which may tend to prove the cause of death and insure that the scene is properly secured.

10. If the deceased resides in Nebraska, attempt to contact the deceased's primary care physician, physician assistant, or nurse practitioner and obtain the following information:

a) Will the physician, physician assistant, or nurse practitioner, sign the death certificate.
b) If so, what will he/she list as the cause of death
c) This information may be obtained by medical staff on the scene (e.g. Hospice Nurse)

B. After the road patrol supervisor or his or her designee have completed all applicable steps, he or she shall determine if the death was a result of natural causes. If the road patrol supervisor is unable to make this determination, the on-call investigator shall be contacted and shall respond to the scene and complete a death investigation (see I-3300). The following items may be used to make the determination that a death investigation needs to be completed:

1. The death was unattended.
2. The victim had not been seen by a physician for an extended period of time.
3. A physician, physician assistant, or nurse practitioner will not sign the death certificate
4. The victim has no significant medical history.
5. The coroner investigation reveals questionable circumstances surrounding the death.
6. The coroner may enlist the aid of any qualified physician present in determining the correct action; however, this does not relieve the coroner from making the final determination of whether or not a death investigation would be appropriate.

C. If the road patrol supervisor or his or her designee determines the deceased party was on Hospice, Comfort Care, Palliative Care, or the death was of natural causes and the primary care physician of the deceased is signing the death certificate, he or she shall complete a coroner worksheet. The coroner worksheet shall be turned in no later than the 0800 hours following day. The coroner worksheet shall contain the following:

1. General information regarding the call (times, location, reporting party, etc.)
2. Demographic information for deceased including social security number
3. Name and information for deceased's next of kin
4. Name of physician, physician assistant, or nurse practitioner signing the death certificate
5. Cause of death (as determined by physician)
6. Medical history for deceased
7. List of deceased's medications
8. Time coroner pronounced death
9. Funeral home that will be completing arrangements for transport and disposal of deceased
10. Report number for original call
11. Separate report number for coroner call

D. If there is any question about what type of report shall be taken, the road patrol supervisor or his or her designee shall consult with the on-call investigator about the death investigation. In any case that involves photographs or property taken as evidence, the road patrol supervisor or his or her designee shall complete an incident report.

E. Additional Considerations:

1. **Other Jurisdictions:** When coroner calls are made at medical facilities in Sarpy County and the deceased died as a result of violence or suicide that occurred in another county, only the County Attorney of that county may order an autopsy. The responding coroner shall contact the County Attorney of the county involved and the Sarpy County Attorney and report the circumstances.

2. **Suicides:** A death investigation/autopsy shall be completed on all suspected suicides.

3. **Traffic Crashes:** A death investigation/autopsy shall be completed on the deceased operator of a motor vehicle involved in a motor vehicle crash. A death investigation/autopsy may also be preformed on passengers at the discretion of the assigned investigator. Each victim shall be assigned a separate report number, connecting to one another and the original crash report number (see P-4010 Serious Personal Injury and Fatality Traffic Crashes).

4. **Stillborn Deaths**: A death investigation/autopsy (N.R.S. 23-1824) is not required on a stillborn if:

   a) There has been no sign of breathing since delivery.
   b) There has been no heartbeat since delivery.

5. Only a physician licensed to practice medicine in the State of Nebraska may sign the death certificate.

6. **Prescription Medications(s):** In death investigations not requiring an autopsy or further investigation, the assigned deputy may offer to the family of the deceased, assistance with the destruction of any prescription medications or controlled substances belonging to the deceased. The destruction of the medication is voluntary and shall be completed on site (e.g. home of the deceased) via drug destruction system. After the medications(s) are destroyed, the destruction bag and its contents shall be left with the family.